

OCCUPATIONAL HEALTH RISK FACTORS AND IMPLICATIONS ON WELL-BEING AND FAMILY DYNAMICS AMONG P'URHEPECHA MEXICAN MIGRANTS IN THE UNITED STATES

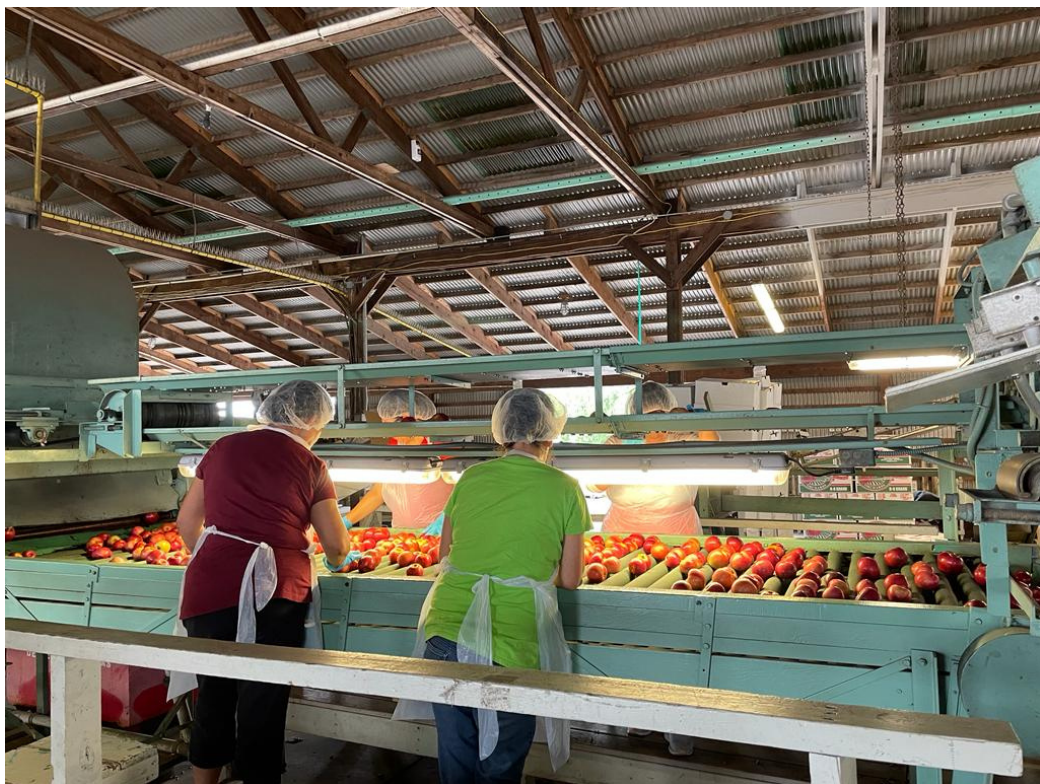
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INTRODUCTION

The P'urhépecha, an indigenous group centered in the state of Michoacán (Mexico), has been engaged in a century-long migration to the United States (Castillo, 1988). This international mobility has been driven by the pursuit of new and better employment opportunities to cover the basic necessities, such as access to food, housing, health, clothing, footwear and education. Despite the long migratory history of the P'urhépecha, this group has been overlooked within the literature specialized in migration issues. Studying this indigenous group provides valuable insights into different and distinct experiences lived compared to other larger migrant groups such as Latin and Mexican migrants. The **objective of this research** is to analyze the labor conditions experienced by Mexican indigenous migrants in the United States. Furthermore, it seeks to delve into the personal implications on their health and well-being, as well as the dynamics within their families. The **methodology** employed in this study relies on the ethnographic method, incorporating participant-observation, in-depth interviews and informal talks with P'urhépecha migrants living in the metropolitan area of Chicago and the rural-southern region of the state of Illinois.



JOB NICHES

Following the occupational insertion patterns of Latino migrants and other minority groups, Mexican indigenous individuals quite often engage in employment sectors commonly known as the '3Ds' because they are characterized as dirty, dangerous and difficult due to the conditions and the nature of the work itself (Schenker, 2011). These labor sector include the following industries:



The analysis of the work trajectories of the P'urhépecha shows that many of them work up to two or three jobs (in different sectors) simultaneously to cover the needs in their places of origin and residence.

JOB CHARACTERISTICS



Informality and subcontracting: Employment arrangements often lack formal written contracts.
Insecurity and instability: Unexpected and unjustified dismissals.
Low salaries: Many of these migrants receive the minimum wage, forcing them to work overtime.
Lack of Social Security coverage: The majority do not have health insurance or paid vacation or sick leave insurance.



Lack of preventive measures: Employers often fail to provide adequate safety materials or training.
Exposure to toxic substances and materials: Workers are often exposed to hazardous materials like agrochemicals, metals and paints.
Exposure to extreme climates: Outdoor labor exposes workers to temperatures ranging from -12° C to 40° C.
Long work days: Working hours of up to 12 hours a day.

HEALTH DAMAGE

The precarious working conditions endured by P'urhépecha immigrants expose them to occupational risk factors significantly impacting their physical and mental health. Observed effects in the migrant P'urhépecha population include:

Cronic Degenerative Diseases	• Diabetes • Cancer • Renal insufficiency
Musculoskeletal Injuries	• Arthritis • Osteoporosis • Spine injuries
Disabilities	• Physical • Mental • Organic (Blindness, for example)

The lack of medical coverage, including preventive and routine treatments, due to no work benefits, has hindered the adequate treatment of conditions, causing severe complications over time. These complications require expensive procedures, such as dialysis or hemodialysis. Health complications, lack of medical coverage, and weak support networks are some of the factors that influence migrants deciding to return to their communities of origin in Mexico.

CASE STUDY: DISRUPTED FAMILY DYNAMICS

In January 2021, Andrea, an undocumented migrant living in the USA for 20 years, had a terrible car accident following a strenuous and exhausting workday. The incident left Andrea immobilized in both limbs and inflicted severe injuries. Given her undocumented migrant status, she does not have access to health insurance or a social security number, making things more challenging for her and her family. **Family dynamics have shifted dramatically** after the accident. Previously, Andrea and her husband, Isidro, shared the responsibility of being the family's financial providers. However, following the incident, Isidro has become the sole breadwinner. Marisol, the eldest daughter, is now in charge of housework, looks after her young siblings and assists her mother in caregiving duties. **Regarding education:** Marisol had to discontinue her studies. Prior to the incident, she was in her final year of high school.

FINAL THOUGHTS

Currently numerous P'urhépecha migrants grapple with diverse diseases and/or disabilities stemming from their work experiences in the United States. These health changes make it difficult for them to carry out their usual daily and cultural activities. A number of sick and/or disabled migrants are compelled to return to their communities of origin without any health insurance or any support. These types of problems have been left out of official statistics and government policies, which is why it is relevant to monitor and study the living conditions of this population. By shedding light on the daily challenges experienced by migrants in their workplaces, along with the adverse effects that often extend to their family dynamics, we can initiate a collective awareness about the absence of inclusive policies that prioritize the well-being of these migrants and their families.

BIBLIOGRAPHIC REFERENCES

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Schenker, Marc (2011). *Migration and Occupational Health: Understanding the Risks*. In: Migration Information Source. Journal of the Migration Policy Institute.

